

PACKET — SIGUE MX

Week 5 · Chapter 4: Contactless Love · Role: ADVERSARY

Product concept: SIGUE MX — Del resultado al siguiente paso.

1. Problem / Vacuum

Mexico's early-detection problem is not only whether a screening tool can identify risk. The critical vacuum is what happens after a positive result. A person can receive useful information and still be left alone inside a fragmented, costly and confusing health system. SIGUE MX attacks the **Detection** → **Treatment Navigation** vacuum.

2. Adversarial thesis

Early detection is not automatically health value. If a system detects risk but does not help the person understand the result, identify a realistic next step, prepare for the visit, and recover when access fails, better data can become an information liability.

3. MVP

A simulated, Spanish-language navigation assistant that takes a screening result and converts it into a structured next-step plan. It explicitly distinguishes screening from diagnosis, explains uncertainty in plain language, surfaces public/low-cost route options as simulated examples, prepares the user for an appointment, and captures whether the next step happened or where access broke.

4. What it is NOT

Not a symptom checker. Not a diagnostic tool. Not a treatment recommender. Not a provider directory claiming live availability. Not a medical decision-maker. The MVP uses simulated resource data and clearly labels it as such.

5. Core user flow

Landing → enter screening result/context → receive plain-language interpretation → choose a realistic route → appointment-prep checklist → weekly follow-through signal → barrier captured → alternate route suggested.

6. Data model

ScreeningResult: type, category, date, municipality, insurance, time availability, transport constraint, cost constraint. Guidance: interpretation, uncertainty note, next-step options, questions to ask. ResourceOption: route type, cost class, accessibility notes, simulated flag. FollowUp: status, barrier, notes, createdAt.

7. Design constraints

No diagnosis; no false certainty; no invented prices or availability; no real personal data in the MVP; accessible alternatives; clear uncertainty; user agency; safety-oriented language; simulated resources labeled visibly.

8. Global → local benchmark

NHS Healthier You Diabetes Prevention Programme demonstrates a pathway from risk identification into a structured prevention program. Unite Us demonstrates closed-loop referral tracking from referral through outcome. Mexican startup PROSPERiA/retinIA demonstrates that AI screening can already reach people through non-hospital channels. SIGUE MX combines the lesson: detection needs a downstream pathway, and the pathway must fit Mexico's access constraints.

9. Shadow / harm prevention

The chapter's shadow is contactless care that feels efficient but leaves people alone. SIGUE MX prevents this by making interpretation and follow-through first-class product steps rather than treating detection as the finish line.

10. Success criteria

A user can understand that a screening result is not a diagnosis; identify at least one realistic next step; see what to bring/ask at the next appointment; report an access barrier; and receive an alternative route without the product pretending to verify medical status or live availability.